

Course Quality Evaluation: EMDR with Complex Trauma and Structural Dissociation (EMDR ARABIA)

Status: DRAFT. AI-assisted evaluation for review by the course director.

Date: 24 September 2026

Framework: the eight criteria in the evaluation prompt supplied by the course author.

Companion documents: course-scientific-review.md and fidelity-readiness-audit.md.

Verdict: the course is well designed, accurate and safe. It is held back by how learning is measured and by limited hands-on practice. Criteria 4 and 8 can only be partly judged, because no observation of the trainer and no learner feedback were available.

The four participant workbooks (supplied after the audit; closes audit defect F-07)

- **No leaks:** they contain no case facts and no answers.
- **Timings match the slides:** all 21 tasks carry the same minutes as their slides. The optional task (Task 11, in block 3.1) is marked optional.
- **Files and right-to-left formatting are sound:** all four open, and all 48 tables are set right to left.
- **One gap:** Days 2 and 4 don't carry the "no one is required to share anything personal" line that Days 1 and 3 have. The participant manual has it every day, but repeat it in the workbooks.

1. Clarity of learning objectives: good, not fully SMART

- **Strengths:**
 - There are 42 objectives. Most are specific and observable: distinguish, classify, justify, decide.
 - Each is tied to a 60-minute block, so the timeframe is built in.
- **Weaknesses:** four objectives can't be measured:
 - 2.2: «تفهم» (understand)
 - 2.4: «تتعرّف» (become familiar), twice
 - 4.4: «تعرف» (know)

2. Content relevance and accuracy: strong

- **Checked:** 48 claims against their sources. 40 are supported, and 3 were confirmed by the course author.
- **Needs fixing:**
 - two outdated claims: the suicide-risk "predictor" wording;
 - one overstated claim: the PTSD→DID spectrum;
 - one misquote: the RDI evidence;
 - one citation error: IHD.
- **Retractions:** none of the course's sources have been retracted.

3. Instructional design: a clear strength

- Every block follows the same order: explanation → skill → worked example → case task → summary.

- Two cases run across all four days and are revealed step by step; all 22 scheduled facts appear on time.
- Every block is exactly 60 minutes, and about 40% of the time is lecture.
- **Weaknesses:**
- Block 1.2 is 40 minutes of lecture with only a poll.
- The order used conflicts with the rule marked "approved" in structure v2, which puts the skill first.

4. Delivery and engagement: can't be judged fully

- **What the design supports:**
- several ways to take part: chat, mic, polls, individual writing, exit questions;
- a named wellbeing assistant with a breakout room;
- content warnings before sensitive reveals.
- **What's missing:**
- The trainer's delivery can't be judged without observing a session or seeing learner feedback.
- All tasks are individual writing, with no pairs or small groups, which may lower energy over a 5-hour online day.

5. Learning materials: strong

- The trainer manual, 314 slides, run sheet, case record, participant manual and workbooks all match each other.
- Each file has a clear job: the participant manual explains concepts, the workbooks hold response space, and answers live only in the trainer manual.
- **Weaknesses:**
- Slides are in Egyptian colloquial Arabic while the manuals are in Modern Standard Arabic; confirm that suits a pan-Arab audience.
- The slide decks have no DRAFT marking or AI disclosure.
- The visual layout has not been checked.

6. Assessment and feedback: the weakest area

- **Strengths:**
- All 21 tasks have model answers and a list of "correct it if you hear this" errors.
- Trainer comment follows every share-out.
- An exit question closes each day.
- **Weaknesses:**
- A 12-item pre-test and post-test can't cover 42 objectives, and the test was not supplied for review.
- Three objectives have no task that assesses them: 1.3, 2.4, and 3.1, whose only task is optional.
- The course measures only knowledge, not changes in clinical behaviour.

7. Practical application: good for decisions, thin for skills

- **Strengths:**
- Every task is a realistic clinical decision.
- Participants write assessment questions in their own words (Task 5).
- The final task runs one case from start to finish.
- **Weaknesses:**
- There's no practice with feedback on what the therapist actually says. The demonstrations are read by the trainer, and 0 minutes are directly observed.

- Two core actions are named but not taught: orienting a client back to the present, and building a safety plan.

8. Overall impact: likely high for its stated purpose, but unproven

- **Purpose fit:** it's a decision-making course, and the design fits that purpose well.
- **Evidence of impact:** there isn't any yet. No test results or learner feedback were available.

Recommendations

1. **Fix the five accuracy items and rewrite the four objectives** before the first run. These are small text edits, set out in the audit's defect log.
2. **Add one short skills practice per day.**
3. Format: 10–15 minutes in pairs, with fictional cases and an observer checklist.
4. Examples: wording a stop signal, orienting a client to the present, and asking the Day 2 risk questions aloud.
5. This turns decisions into things participants can actually say in a session.
6. **Rebuild the assessment.**
7. Map the test items to objective IDs and cover at least the 20 core objectives.
8. Add one scored case vignette on Day 4, marked with a rubric.
9. Collect a 3-month follow-up on what participants now do differently in practice.
10. **Add the two missing teaching slides:** what goes in a safety plan (from TTRD Appendix C) and wording for orienting a client to the present.
11. **Break up block 1.2** with a short classification task in place of 10 minutes of lecture.